

KESTREL HEALTHCARE

Department of Ophthalmology
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Portland, Maine 04101
(207) 555-0251

COMPREHENSIVE OPHTHALMOLOGY EXAMINATION REPORT

Patient: Joanne Lawton
Date of Birth: December 3, 1950
Age: 74
Address: 47 Atlantic Street, Portland, Maine 04101
Phone: (207) 555-0134
Medicare ID: 1EG4-TE5-MK72
Referring Provider: Robert Chambers, MD, Greenleaf Health Portland
Examining Provider: Dr. Stephen Patterson, MD
Examination Date: November 14, 2025
Visit Type: Established patient, comprehensive examination with dilation

CHIEF COMPLAINT AND HISTORY

Patient is a 74-year-old woman presenting for annual comprehensive eye examination. Patient reports no significant change in vision since last examination (November 2024). Reports occasional difficulty reading small print in low light, particularly menus at restaurants and fine print on medicine bottles. No complaints of eye pain, redness, floaters, flashes, or sudden vision change. No headaches related to vision. Patient is driving locally in Portland and reports no difficulty.

Past ocular history: Bilateral cataracts, first noted 2018, right eye more advanced than left. Monitored annually.
Systemic history pertinent to ocular health: Type 2 Diabetes (managed, A1c 7.4% as of last primary care visit), Hypertension (managed).
Current medications: See referring provider records. No ocular medications.
Allergies: None documented.

VISUAL ACUITY (best corrected)

Distance:
Right eye (OD): 20/40 (decreased from 20/30 in November 2024)
Left eye (OS): 20/25 (unchanged from November 2024)
Both eyes (OU): 20/30

Near vision (J notation, best corrected):
Right eye (OD): J4 (fair; mildly diminished from J3 last year)
Left eye (OS): J2 (good)

Patient's current glasses prescription noted; refractive correction done by outside optometrist.

INTRAOCULAR PRESSURE

Right eye (OD): 15 mmHg (normal: under 21)
Left eye (OS): 14 mmHg (normal: under 21)
Method: Non-contact tonometry

EXTERNAL AND ANTERIOR SEGMENT EXAMINATION

Lids and lashes: Normal. No blepharitis. Good lid closure.
Conjunctiva: Clear, no injection.
Cornea: Clear. No corneal edema, opacity, or abnormalities.
Anterior chamber: Deep and quiet, no cells or flare.
Iris: Normal, round, reactive to light bilaterally.

Lens examination (post-dilation):

Right eye (OD): Nuclear sclerosis grade 2.5 to 3 out of 4 (moderate to moderately advanced). Posterior subcapsular changes, grade 1. Increased from grade 2 at last examination.

Left eye (OS): Nuclear sclerosis grade 1.5 to 2. Trace posterior subcapsular change. Stable from last examination.

Summary: Bilateral cataracts. Right eye more advanced. Left eye mild and stable.

DILATED FUNDUS EXAMINATION

Both eyes examined with indirect ophthalmoscopy following dilation with tropicamide 1% and phenylephrine 2.5%.

Vitreous: Clear bilaterally. No vitreous hemorrhage. No posterior vitreous detachment noted.

Optic disc:

Right eye: Cup-to-disc ratio 0.3, sharp margins, pink healthy appearance.

Left eye: Cup-to-disc ratio 0.3, sharp margins, pink healthy appearance.

No disc edema, pallor, or cupping concerning for glaucoma.

Macula:

Right eye: Flat and even foveal reflex. No macular edema. No drusen. No changes concerning for diabetic maculopathy.

Left eye: Flat and even. Normal.

Diabetic retinopathy screening:

Right eye: No diabetic retinopathy changes identified. No microaneurysms, dot hemorrhages, cotton-wool spots, or neovascularization.

Left eye: No diabetic retinopathy changes identified.

Overall: No diabetic retinopathy. This is consistent with well-managed diabetes, though A1c elevation noted in records. Continue annual diabetic eye screening.

Periphery:

Right eye: Normal, no tears, breaks, or lesions.

Left eye: Normal.

IMPRESSION

1. Bilateral cataracts. Right eye: moderate to moderately advanced nuclear sclerosis with posterior subcapsular cataract; visual acuity 20/40 corrected, decreased from 20/30. Left eye: mild cataracts, stable.
2. No diabetic retinopathy. Recommend continued annual screening given diabetes diagnosis.
3. Intraocular pressure normal bilaterally. No glaucoma findings at this time.

PLAN AND RECOMMENDATIONS

Right eye cataract: The reduction in best-corrected visual acuity from 20/30 to 20/40 in one year represents meaningful progression. Patient's ability to drive locally remains adequate at this level, however we are approaching the threshold at which surgery becomes both clinically appropriate and practically beneficial for daily function.

Recommendation: Surgical consultation in 2026 if vision continues to decline or if patient experiences significant functional limitation (nighttime driving difficulty, reading impairment, difficulty with daily tasks). Target timeline: surgical evaluation by November 2026 or sooner if symptomatic.

Discussed with patient: Patient is aware of the right-eye timeline. She is not anxious about surgery but prefers to wait until the functional impact is clear. She agreed to contact the office if her driving confidence diminishes or if she has difficulty reading medication labels.

Left eye cataract: Stable. No surgery indicated at this time. Continue annual monitoring.

Diabetic eye health: Patient counseled on importance of A1c control for ocular health. Diabetic retinopathy risk increases with poorer glycemic control. Current absence of retinopathy is encouraging; continue annual dilated exams.

Next scheduled examination: November 2026 (annual comprehensive with dilation).

Emergency contact: (207) 555-0251 during office hours; go to MaineHealth emergency department for acute vision changes.

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Date: November 14, 2025